

development RURAL HEALTH SCHEMES

BIRTH OF HOPE

A series of health initiatives undertaken in Madhya Pradesh—Janani Suraksha, Janani Express and Matri Shakti—ensure better care of a pregnant woman and her child

■ By Ambresh Mishra

Your money or your lives: sounds like a standard cliché from a Spaghetti Western or a Bollywood bandit movie from the 1970s and '80s. But in Madhya Pradesh, it was a doctor whose callous words had the same effect as that of a gun-toting brigand in reel life. Till some years ago when a harried villager with his pregnant wife would come to an unscrupulous private nursing home for her delivery, taking advantage of his helplessness, the doctors would grimly inform him about the condition of his wife and unborn baby and tell him that only one of them could be saved. The cost of their lives: money. The hapless villager had no other option but to pay, even if it meant interminable loans or sale of land, to save his wife and the baby.

According to official figures in 2004, almost three-fourths (74 per cent) of the total deliveries in the state took place outside medical institutions, in dangerous circumstances under the guidance of ill-trained midwives. As a result, maternal mortality rate was 498 per lakh and infant mortality rate was 76 per thousand. The figures were the highest for any state in the country and according to an official survey, below average ratings on nearly all human development indices confirmed the status of Madhya Pradesh as a sick state. However, the launch of two major ini-

tiatives—Janani Suraksha and Janani Express—has helped bring about a change. While maternal mortality rate is down to 354, infant mortality rate has declined to 66, as per 2008 figures.

Thanks to these government initiatives a pregnant woman, enrolled under Janani Suraksha which is run under the National Rural Health Mission (NRHM), doesn't need to pay harried visits to unscrupulous nursing homes for her delivery. Better healthcare is just a call away. Once she goes into labour, her relatives or an anganwadi worker call a helpline for an ambulance. The call centre directs the nearest ambulance to reach her door and take her to a government hospital or a certified nursing home for safe institutional delivery. And for all this, the woman and her family do not have to pay anything. Rather they get paid Rs 1,400 in cash while the accompanying *prerak* (handler), usually an anganwadi worker, gets Rs 350 soon after she is discharged from the hospital and another

Rs 250 is kept aside for her transport.

In another unique public-private-partnership initiative, private nursing homes have been roped in to make up for the shortage of government doctors. The registered maternity homes, 118 in the state, are given Rs 1 lakh each in advance to deal with maternity cases. They are paid Rs 1,200 each for normal and Rs 5,500 for caesarean deliveries. A proposal has been made to hike the remuneration to Rs 1,500 and Rs 7,000, respectively, to attract more private institutions to the scheme's fold. Private vehicles have been engaged under Janani Express, an ambulance scheme for pregnant women, in 270 out of 313 development blocks of the state. Initially, the numbers of the ambulances were provided to anganwadi workers for emergency, but last year, the health department set up a call centre in Bhopal to coordinate the entire service. The call centre works for Bhopal and its adjoining districts



Photographs by PANKAJ TIWARI